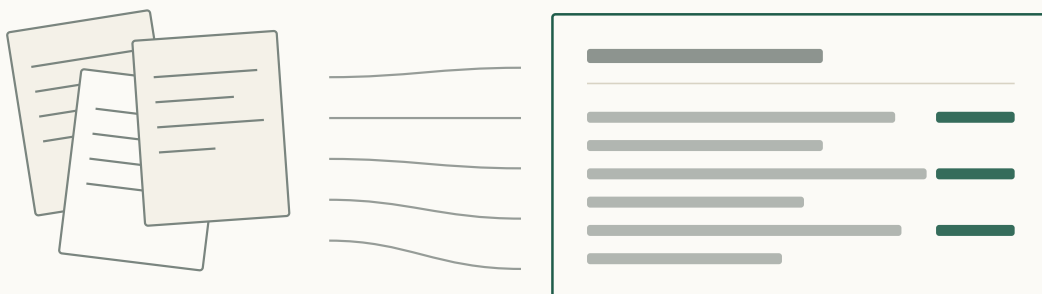

Get your evenings back.

Turn ten years of paper files into a searchable patient record — without your receptionist retyping a single one, and without you ever wondering if the system put something on the wrong patient.



Built specifically for SA practices — medical aid schemes, ICD-10, NAPPI, ZAR billing. Not an international tool bent to fit.

SEE IT FIRST
ON YOUR OWN FILES.
TWENTY MINUTES.

You see 30 patients a day. Their notes live in three places — the paper file on your desk, the script pad, a sticky note from reception. *At 7pm you're still typing, still hunting for that one diagnosis, still copying ICD-10 codes off a list so medical-aid claims don't bounce.* Somewhere in the back room, ten years of patient files sit in cabinets nobody can search.

You already know the problem. We're not going to oversell it.

Going digital today means one of two bad options: someone retypes every record by hand — months of admin — or a third party charges a fortune to only scan your records into PDFs. So most practices stay on paper, or land in a half-paper-half-digital limbo that's worse than either. And the tools that promise to fix it tend to make the doctor's day slower, not faster.

We fix the back room first. Here is what it actually does — three things, plainly.

1 Stop retyping. Stop paying someone to retype.

Your receptionist scans into the same folder they already use. Nothing changes in their day — no new software, no “did you remember to upload that.” By the time you log in, the records are read, structured, and waiting for a 30-second check. The months of migration admin simply don't happen.

2 Find anything, in seconds, across every file you've ever had.

Ask a question in plain English and get an answer — with the actual scanned page one click away. Ready when a 2014 patient walks back in, when a medical aid disputes a claim from three years ago, when you need it and the answer is buried in a shelf of folders.

`ask: which of my diabetics haven't been in for six months?`

3 It catches what falls through the cracks — before it's a problem.

The warfarin patient overdue for an INR. The pre-authorisation that expires next week. The specialist referral that never came back. The system surfaces these for you each morning — before they become the thing you wish you'd caught.

You've been promised things before. So before anything else, the three questions you're actually asking — answered straight.

“Will it put something on the wrong patient?”

No record enters a patient's file until you confirm it's the right patient. The system shows you the match; your staff says yes or no. Wrong-patient mistakes are caught at that gate — before they happen, not discovered months later.

“Could the AI just be confidently wrong?”

The system extracts; a person approves. Your receptionist knows your handwriting. They see what was found, see how confident the system is on each field, and correct anything wrong before it counts — about 30–60 seconds per document. A person is always the final word.

“If I'm audited or have to defend a decision, am I covered?”

Every clinical action on a patient is logged — every approval, every correction, every reversal, with who did it and when. If a medical aid scheme audits the practice, or you need to show the basis of a decision, the history is one query away rather than a week of digging through boxes.

These aren't claims layered on afterwards. The safety gate, the human-approval step, and the logged history are how the system is built underneath — because the people who built it have shipped clinical software before, and know that the first wrong-patient record is the moment a practice stops trusting a system forever.

The reason practices stay on paper isn't that digitising is expensive in principle. It's that it arrives as a project bill no practice wants to sign. So we don't price it as a project.

THE QUOTE NOBODY SIGNS

A once-off capital project

Standard digitisation of a established practice's archive is quoted as a large one-off bill — the kind of number that gets a project shelved before it starts.

WHAT WE DO INSTEAD

From R 3,500 per month

A predictable monthly subscription. The archive becomes searchable structured data month by month — no project sticker shock, no capital approval, opex not capex.

FOUNDER PRICING

Locked for 12 months for early practices. Available until 31 August 2026.

EXPORT INCLUDED

Your cleaned records, out via CSV or the standard healthcare format. No extra charge.

NO LOCK-IN

Month-to-month. No setup fees. Cancel with 30 days' notice.

NO IT PROJECT

Runs in your browser. No installation, no consultant, no server.

FROM A PRACTICE WE WORK WITH

A two-doctor practice, open since 2010, had roughly 20,000 patient files at about ten pages each. The standard once-off digitisation quotes they were given ran to around R400,000 — the kind of project bill that gets shelved. On a monthly subscription, the same archive becomes searchable structured data month by month, at a predictable cost the practice approves without a capital sign-off.

KEEP YOUR PAPER NOTES

We digitise what you already do. You don't change how you consult or what you write.

KEEP YOUR WORKFLOW

A layer that adds capability, not a system that replaces one that works. Reception learns nothing new.

KEEP YOUR EXISTING SYSTEM

If you already run an EHR, this works alongside it — ingest and export via CSV or the standard healthcare format.

BUILT FOR HERE

Medical aid schemes, SA ICD-10, NAPPI codes, ZAR billing. Built for it, not adapted to it.

THE ONLY THING WE'RE ASKING FOR

Bring us your messiest shoebox of files.

We won't ask you to take anything on faith. Bring the worst, most disorganised stack of records you have — the ones you'd least want to digitise by hand. We'll show you, on your own files, in about twenty minutes, exactly what comes out the other side. Then you decide.

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